

mCRPC Radioligand Therapy — 2026 ASCO/AUA Data Releases

Mid-2026 Clinical Analysis — Alpha vs. Beta Emitters, Sequencing, and the Emerging Competitive Landscape
July 2026

Alpha-Emitters and Earlier Lines Reshape mCRPC RLT in 2026

225Ac Salvage

- **100%** PSA50 in RLT-naïve patients and **56.3%** PSA50 post-Pluvicto with 225Ac-PSMA-617 (AcTION Phase 1) [1]
- 225Ac-rosopatamab delivered **8.4-month** rPFS after 177Lu failure vs an SOC benchmark of only ~2–4 months (CONVERGE-01) [2]
- Recommended AcTION dose: **10 MBq q8w**, establishing a new salvage benchmark [3]

RLT into mHSPC

- PSMAAddition: adding Pluvicto to ADT + ARPI reduced radiographic progression risk by **28%** (HR **0.72**) in mHSPC [4]
- Interim OS trend (HR **0.84**) has not reached statistical significance, tempering widespread mHSPC adoption [5]
- Grade 3+ anemia rose from **14%** (SOC) to **28%** (RLT+SOC), reflecting intensification cost [5]

Sequencing Evidence

- PRECISION real-world data: Pluvicto after exactly one prior ARPI delivered **15.8-month** mPFS vs **12.7 months** after multiple ARPIs [6]
- PSMAfore: RLT doubled taxane-naïve mPFS to **12.0 months** vs **5.6 months** on ARPI switch [7]
- VISION post-taxane mOS improved from **11.3** → **15.3 months** on 177Lu-PSMA-617, anchoring late-line use [8]

Competitive Pipeline

- TLX591 (Telix): antibody-based 177Lu with a **2-dose, 14-day** fractionated schedule vs Pluvicto's 6-cycle regimen [9]
- PNT2002 (Lilly/Point): SPLASH trial met primary endpoint with **9.5 vs 6.0-month** rPFS; mature OS due late 2026 [10]
- Convergent's CONV01-α showed durable anti-tumor activity in Lu-PSMA-exposed mCRPC at ASCO 2026 [2]

Non-PSMA & Safety

- AKY-2519 (B7-H3) Phase 1b initiated Jul 2026 to address the **~30%** of mCRPC patients who do not respond to PSMA therapies [11]
- Xerostomia remains the dominant toxicity for small-molecule 225Ac ligands; antibody-based alpha therapies mitigate this risk [12] [2]
- "Sink effect" at low tumor volume flagged as a normal-tissue radiation concern for potent alpha-emitters [13]

New 2026 RLT Data Resets Efficacy Bar Across Every mCRPC Line

• Clinical benchmarks — mid-2026 readouts; SOC = current standard of care; RLT = radioligand therapy

Metric (Setting)	Current SOC	New Data (2026)	Trial / Agent
Post-ARPI, Taxane-naïve mCRPC			
mPFS (months)	5.6	12.0	PSMAfore (177Lu-PSMA-617) [7]
Post-Taxane mCRPC			
mOS (months)	11.3	15.3	VISION (177Lu-PSMA-617) [8]
ORR (%)	2%	30%	VISION (177Lu-PSMA-617) [14]
RLT-naïve — Alpha-Emitter Frontier			
PSA50 (%)	46% (VISION)	100%	AcTION Ph1 (225Ac-PSMA-617) [11]
Post-177Lu Failure (Salvage)			
rPFS (months)	~2–4 (no clear SOC)	8.4	CONVERGE-01 (225Ac-rosopitamab) [2]
PSA50 (%)	N/A	56.3%	AcTION Ph1 (225Ac-PSMA-617) [11]
Real-World 1L mCRPC (Sequencing)			
mPFS — >1 prior ARPI vs 1 prior ARPI (months)	12.7	15.8	PRECISION Platform (RWE) [6]
Safety — Treatment Intensification (mHSPC)			
Grade 3+ Anemia (%)	14%	28%	PSMAddition (Pluvicto + SOC) [5]

KEY TAKEAWAYS

- **225Ac alpha-emitters** redefine the salvage benchmark: **8.4 mo** rPFS and **56.3%** PSA50 post-Pluvicto, versus no established SOC in this segment [\[2\]](#) [\[1\]](#)
- **PSMAfore** more than doubles mPFS to **12.0 mo** (vs 5.6 mo ARPI switch), positioning 177Lu ahead of chemo in taxane-naïve mCRPC [\[7\]](#)
- Real-world **PRECISION** data confirm **15.8 mo** mPFS when Pluvicto follows exactly one ARPI, a **3.1-month uplift** over multi-ARPI sequencing [\[6\]](#)
- Earlier-line intensification is not free: **Grade 3+ anemia doubles from 14% to 28%** with RLT+SOC in mHSPC, tempering enthusiasm ahead of mature OS [\[5\]](#) [\[4\]](#)

Actinium-225 Emerges as Post-Lutetium Salvage Standard

POST-177Lu rPFS BENCHMARK

8.4 months

CONVERGE-01 (225Ac-rosopitamab) vs. ~2-4 mo prior SOC [\[2\]](#)

AcTION Phase 1 — Efficacy Signal

- **100%** PSA50 and **90%** PSA90 in taxane-naïve mCRPC [\[1\]\[12\]](#)
- **56.3%** PSA50 in prior Pluvicto (177Lu) failures [\[1\]](#)
- RP2D: **10 MBq q8w** of 225Ac-PSMA-617 [\[3\]](#)

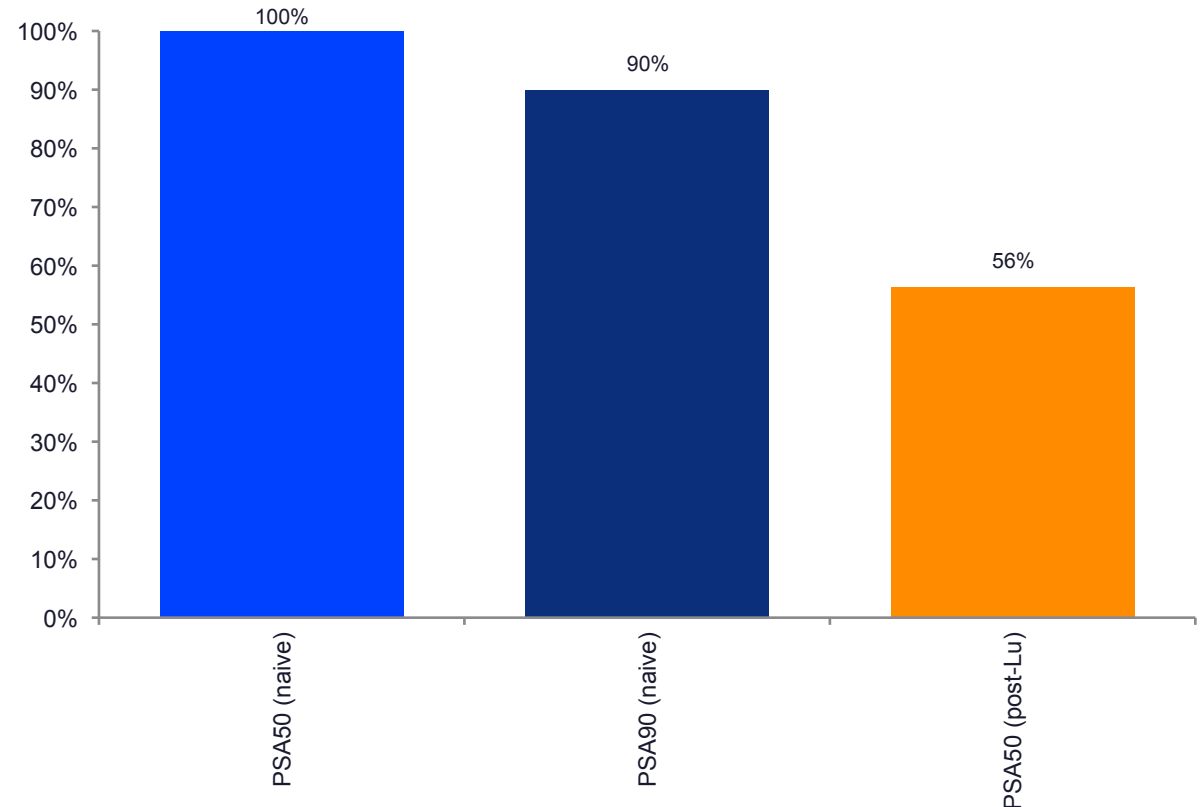
CONVERGE-01 — Durability Post-Lu

- **8.4-mo** median rPFS in 177Lu-exposed mCRPC [\[2\]](#)
- Antibody-based alpha carrier (CONV01-alpha), distinct from PSMA-617 [\[2\]](#)
- Positive Phase 2 readout at **ASCO 2026** [\[2\]](#)

Safety & Delivery Trade-offs

- Antibody 225Ac shows **manageable xerostomia** vs PSMA-617 dry-mouth [\[2\]\[12\]](#)
- "Sink effect" at low tumor volume — sequencing risk for early lines [\[13\]](#)

AcTION PSA Response Rates by Prior Therapy (%)



AcTION Phase 1 single-series response rates. PSA90 post-177Lu not disclosed. VISION 177Lu-PSMA-617 comparator PSA50 = 46% (RLT-naïve) [\[1\]](#)

Pluvicto Moves Earlier as mHSPC Intensification Signal Strengthens

PRECISION Platform RWE (left) | PSMAddition Phase 3 (right)

Setup Timing Matters

- PRECISION RWE shows **15.8 mo** median PFS when Pluvicto follows exactly one prior ARPI vs **12.7 mo** after multiple ARPI lines — a **3.1-month** durability gap [6]
- Confirms the mHSPC push: earlier RLT sequencing preserves benefit before ARPI resistance compounds [6]
- Supports PSMAddition rationale for moving Pluvicto ahead of chemotherapy in hormone-sensitive disease [4]
- Positions Novartis to defend against fractionated (TLX591) and PSMA-I&T (PNT2002) entrants in 1L [9]

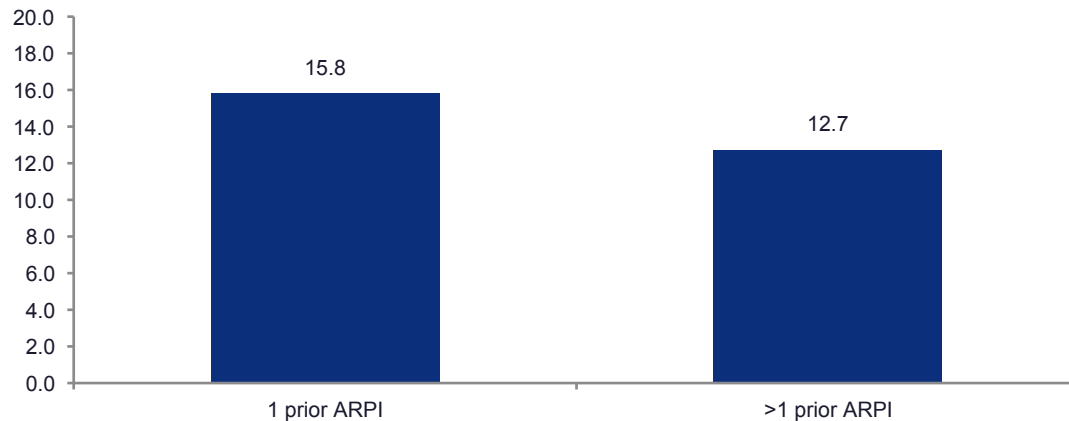
Efficacy vs OS Signal Tension

- PSMAddition delivered a **28%** reduction in radiographic progression risk (rPFS HR **0.72**) in mHSPC [4]
- Interim OS HR of **0.84** has not reached statistical significance, echoing prior ESMO 2023 readout [5]
- ESMO 2023 signal was eroded by **84%** control-arm crossover to Pluvicto after progression [15]
- Discussants at ESMO cautioned against broad mHSPC adoption pending mature OS data [5]

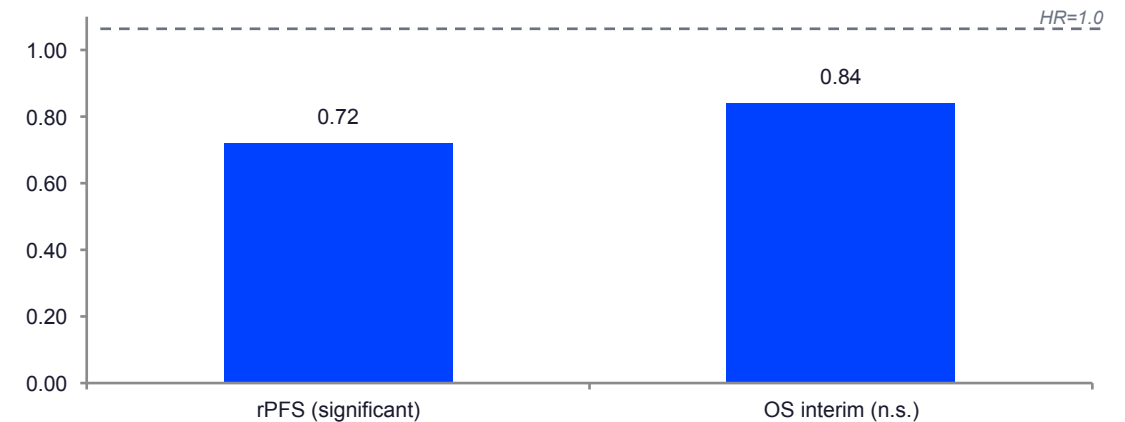
Intensification Toxicity Tradeoff

- Grade 3+ anemia doubled to **28%** on RLT + SOC vs **14%** on SOC alone in PSMAddition [5]
- Earlier-line intensification raises cytopenia burden — must be weighed against rPFS gains [5]
- Sequencing after one ARPI (not multiple) also limits cumulative marrow exposure risk [6]

PRECISION RWE Median PFS by Prior ARPI Lines (months)



PSMAddition Hazard Ratios vs HR=1 Reference



Pipeline Diversifies Beyond Pluvicto With Novel Formats and Targets

Candidate / Sponsor	Isotope & Target	Trial & Key Data	Status / Next Readout
TLX591 Telix Pharmaceuticals 	177Lu beta-emitter; PSMA antibody	ProstACT Global Phase 3 (Part 1); fractionated 2-dose regimen 14 days apart vs. Pluvicto's 6-cycle schedule	Late-breaking Phase 3 Part 1 data presented at ASCO 2026; positioning on patient throughput advantage
PNT2002 Eli Lilly / Point Biopharma 	177Lu beta-emitter; PSMA-I&T small-molecule ligand	SPLASH trial: rPFS 9.5 vs 6.0 months (met primary endpoint)	Mature OS readout expected by late 2026
CONV01-α (rosopatamab) Convergent Therapeutics	225Ac alpha-emitter; PSMA antibody	CONVERGE-01 Phase 2: rPFS 8.4 months in post-177Lu failure setting; reduced xerostomia vs. small-molecule ligands	Positive Phase 2 data presented at ASCO 2026
225Ac-PSMA-617 Novartis 	225Ac alpha-emitter; PSMA-617 ligand	AcTION Phase 1: 100% PSA50 RLT-naïve; 56.3% PSA50 post-Pluvicto failure; RP2D 10 MBq q8w	Phase 1 dose established; xerostomia remains dominant tox
AKY-2519 Aculeus Therapeutics	B7-H3 RLT (payload undisclosed)	Phase 1b initiated July 2026; addresses ~30% of mCRPC patients who do not respond to PSMA-targeted RLT	Phase 1b enrolling — first non-PSMA RLT for prostate

Bull-Bear Debate: Efficacy Gains Collide With OS and Toxicity Risks

Bull Case		Bear Case	
1	225Ac delivers universal PSA50 in RLT-naïve patients 225Ac-PSMA-617 achieved 100% PSA50 and 90% PSA90 in taxane-naïve mCRPC vs 46% PSA50 historical VISION benchmark [12] [1]	1	PSMAddition OS not statistically significant Interim OS HR 0.84 in mHSPC has not cleared statistical significance; discussants urge caution before broad early-line use [5] [15]
2	Alpha rescue works after 177Lu failure 56.3% PSA50 with 225Ac-PSMA-617 in Pluvicto-refractory patients; CONVERGE-01 delivered 8.4-month rPFS vs 2-4 mo SOC [1] [2]	2	Myelotoxicity doubles in earlier lines Grade 3+ anemia rises from 14% (SOC) to 28% when Pluvicto is layered onto ADT+ARPI in mHSPC [5]
3	PSMAddition proves earlier-line RLT expansion Adding Pluvicto to ADT+ARPI in mHSPC cut radiographic progression risk 28% (rPFS HR 0.72) [4]	3	Sink effect risk for 225Ac in low tumor volume Alpha-emitters may irradiate normal tissue when tumor burden is low, raising off-target exposure concerns in earlier-line settings [13]
4	PNT2002 SPLASH meets primary endpoint Lilly/Point PSMA-I&T delivered 9.5 vs 6.0 months rPFS in SPLASH; mature OS due late 2026 [10] [16]	4	Xerostomia remains dominant 225Ac toxicity Severe dry mouth is the leading dose-limiting toxicity for small-molecule 225Ac-PSMA-617 at the 10 MBq q8w recommended dose [12] [3]
5	Real-world 1L data validates early-sequencing thesis PRECISION RWE: 15.8-month mPFS after one prior ARPI vs 12.7 months with multiple ARPIs - supports front-line adoption [6]	5	~30% non-PSMA responder gap still unaddressed Roughly 30% of mCRPC patients do not express sufficient PSMA to respond to RLT; non-PSMA targets (e.g. AKY-2519 B7-H3) only in Phase 1b [11]

Sources

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- [4] Press • Novartis AG • 31 May 26 • "Pluvicto™ demonstrated consistent efficacy across key patient subgroups in metastatic hormone-sensitive prostate cancer"
- [5] JPMorgan Research • Novartis AG • 20 Oct 25 • "Novartis : PSMAAddition detailed data presented showing a 28% rPFS benefit though discussant did not recommend widespread use due to lack of clear OS benefit and toxicity"
- [6] Press • Novartis AG • 24 Feb 26 • "Real-world evidence showed Pluvicto achieved longer PFS when initiated after one ARPI instead of multiple ARPIs"
- [7] CLSA • Novartis AG • 14 Nov 25 • "Novartis - O-PF (Reimagining the future)"
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